Mikhwah General Hospital, Al-Baha
AL-BAHA CLUSTER

DPP	**DEPARTMENTAL POLICY AND PROCEDURE**		
Title	**CHEST TUBE REMOVAL**		
Department	**Neonatology Department**		
Applies to	**All NICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-NICU-016 (v2)**		
Last Version Approval Date	**28/1/2022**	Number of Pages	**3**

	NAME	**POSITION**	**SIGNATURE**	**DATE**
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APPROVAL DATE	**EFFECTIVE DATE**	**TYPE OF REVISION**	**REVIEW DATE**
19/ 3 / 2024	7 / 4 / 2024	TRIENNIAL	18/ 3 / 2027

1.0 PURPOSE

1.1. To ensure safe and consistent practice in the removal of chest tube by a physician assisted by staff nurse.

2.0 DEFINITION/ABBREVIATION

2.1. **Chest tube removal -** is a procedure wherein tube inserted in the pleural space is being removed after re-expansion of the lungs has been attained.

3.0 RESPONSIBILITY

3.1. NICU Physician

3.2. NICU Nurses

4.0 POLICY

4.1. Chest tube must be removed upon physician's order.

4.2. This is a two-person procedure; removal of tube must be done by the physician and a nurse.

4.3. If more than one tube is inserted, the appropriate tube to be removed must be identified by the physician according to chest x-ray film.

4.4. Chest tube must be clamped several hours before removal.

4.5. Strict aseptic technique must be observed.

5.0 PROCEDURE (S)

5.1. Explain the procedure to the parents.

5.2. Administer prescribed pre-medication for pain 30 minutes before procedure.

5.3. Assemble all equipment at bedside.

5.4. Wash hands and wear gloves.

5.5. Prepare dressing to be placed in the chest tube site after removal.

5.6. Remove chest tube dressing. Leave sternal incision dressing intact.

5.7. The physician cuts the suture from the skin and removes the remaining anchoring suture.

5.8. The physician quickly pulls out the chest tube.

5.9. Place Vaseline gauze and 4 x 4 gauze pad into site.

5.10. Apply dressing to the wound site and secure with adhesive tape.

5.11. Have chest x-ray done after removal of the chest tube.

5.12. Discard all supplies including chest tube and drainage bottle in a biohazardous garbage bag.

5.13. Observe patient for signs of respiratory distress caused by loss of negative intra pleural pressure or tension pneumothorax.

5.14. Document all information about the procedure, and patient's response.

6.0 EQUIPMENTS

6.1. Sterile gauze 4 x 4
6.2. Vaseline gauze
6.3. Povidone-iodine solution
6.4. Scalpel
6.5. Adhesive tape
6.6. Chest tube clamps
6.7. Biohazard garbage bag
6.8. Gloves

7.0 ATTACHMENT/FORMS

7.1. Nurses Progress Notes (GDOH-NUR-NPN-203)
7.2. Physician Order Form (GDOH-INP-POF-079)

8.0 REFERENCES

8.1. MOH-KSA Guidelines for Neonatal Care

8.2. Photo Guide of Nursing Skills 2004, by Smith, Duell, Martin

8.3. Lippincott Manual of Nursing Practice, 7th Edition by Nettina

8.4. Fundamentals of Nursing 7th Edition, by Kozier, Erb, Berman, Snyder

8.5. CBAHI Standards NICU 3rd version

9.0 REVISION CLAUSE

9.1. Modification of policy format, lay out and policy coding

KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: ____________ :مستشفى

Region: ____________ :المنطقة/المحافظة

Dept./Unit: ____________ :القسم/الوحدة

MRN: |__|__|__|__|__|__|__|__|__|__| :رقم الملف الطبي

Name: ______________________ :الاسـم

Nationality: ______________________ :الجنسية

Age: __________ ☐ سنة Years ☐ شهر Months ☐ يوم Days :العمر

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ :تاريخ الميلاد

Gender: ☐ Male ☐ Female :الجنس

NURSES PROGRESS NOTES FORM

DATE &TIME	NURSES NOTES (D-data A-action R-response)	REGISTERED NURSE INITIAL WITH ID NUMBER

Note: Write the time in each entry & affix your initial at the end of each paragraph. Document your complete Name, Initial, Job number, Date & Time at the closure of your documentation. Draw a line across empty spaces.

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Date of Birth: ____/____/ 14____ H ____/____/ 20____ :تاريخ الميلاد

Gender: ☐ Male ☐ Female :الجنس

PHYSICIAN ORDER FORM

ALLERGIC TO:

DATE	TIME	PHYSICIAN ORDERS	PHYSICIAN STAMP / SIGNATURE	RN INITIAL / JOB NUMBER

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